Hybrid CNN-LSTM and Physics-Informed Architectures for Real-Time Tumor Ablation Monitoring on Edge Devices

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***Abstract*—This paper presents a comprehensive study on estimating temperature dynamics from ultrasonic video sequences using deep learning models tailored for edge deployment, specifically for monitoring temperature during heat treatment of tumor tissue. We propose a specialized CNN-LSTM architecture for this task and compare it against Pretrained ResNets, Kolmogorov–Arnold Network (KAN) heads, Physics-Informed Neural Networks (PINNs), and Liquid Time Constant (LTC) variants in terms of accuracy and computational efficiency. To enhance robustness and provide a transparent monitoring system, we integrate Uncertainty Quantification (UQ) via Deep Ensembles and Bayesian Neural Networks. Furthermore, we propose a lightweight preprocessing pipeline incorporating dense optical flow to capture temporal dynamics explicitly. We benchmark these models on a simulated edge environment, demonstrating that our optimized architectures achieve real-time performance (>30 FPS) on low-cost, resource-constrained devices like the Raspberry Pi 4 and Jetson Nano, while maintaining high predictive accuracy.**

***Index Terms*—Video Regression, Physics-Informed Machine Learning, Edge Computing, Bayesian Neural Networks, Optical Flow, Thermal Ablation, Medical Imaging**

I. Introduction and Motivation

Thermal ablation has become a widely adopted minimally invasive treatment for solid tumors in the liver, lung, kidney, and other organs [1], [2]. Among ablation modalities, microwave ablation (MWA) is increasingly favored over radiofrequency ablation (RFA) because it achieves faster heating, higher intratumoral temperatures, and larger, more predictable ablation zones with reduced sensitivity to heat-sink effects from nearby vasculature [3], [4]. Technically successful ablation requires complete coverage of the tumor plus a circumferential safety margin of 5–10 mm [2], and tissue coagulation reliably occurs only when temperature is sustained in the 50–60°C range for several minutes, while temperatures above 100°C cause vaporization and carbonization [1]. This narrow therapeutic window makes precise, continuous temperature monitoring essential to avoid both incomplete treatment and collateral damage to healthy tissue.

Estimating physical parameters, such as temperature, from medical imaging data is a critical challenge in non- and minimally-invasive thermal ablation therapies. In procedures like High-Intensity Focused Ultrasound (HIFU), RFA and MWA, precise temperature monitoring is essential to ensure tumor destruction while preserving healthy surrounding tissue. However, direct temperature measurement is often impossible or requires invasive probes such as thermocouples or fiber-optic sensors, which are accurate but sample only discrete spatial locations and add procedural complexity [5]. In this work, we explore the feasibility of using ultrasonic video sequences to regress temperature values, leveraging temporal coherence and visual cues present in the footage.

A major constraint in clinical deployment is data privacy. Transmitting sensitive patient data to cloud servers for processing poses significant security risks and regulatory hurdles. Therefore, inference must be performed locally on edge devices. Furthermore, to ensure widespread accessibility, these devices must be cost-effective and easily retrofittable to existing ultrasonic machines. This necessitates models that are not only accurate but also lightweight enough to run on low-cost hardware like the Raspberry Pi or Jetson Nano.

Finally, for clinical adoption, black-box predictions are insufficient. We emphasize the need for a transparent monitoring system by integrating Uncertainty Quantification (UQ), providing clinicians with confidence intervals alongside temperature estimates.

We address these challenges by:

- Proposing a custom, lightweight CNN-LSTM architecture

optimized for ultrasonic video regression on fast edge hardware.
- Evaluating physics-inspired training objectives that combine data error with reduced bioheat residuals, image-flow terms, and spatial smoothness priors.
- Evaluating Kolmogorov–Arnold Network (KAN) heads and Liquid Time Constant (LTC) temporal models to capture interpretable nonlinear dynamics and continuous-time heating behavior.
- Incorporating probabilistic modeling through Deep Ensembles, Bayesian Neural Networks, and Bayesian Physics-Informed Neural Networks (B-PINNs).
- Benchmarking end-to-end deployment speed using ONNX export and ONNX Runtime across simulated Raspberry Pi 4, Jetson Nano, and Orin Nano profiles.

II. Related Work

A. Non-Invasive Temperature Monitoring

The gold standard for non-invasive temperature monitoring is Magnetic Resonance Thermometry (MRT), which offers high spatial resolution and accuracy. However, MRT is expensive, immobile, and requires complex infrastructure, limiting its availability [5]. Computed tomography can delineate the ablation zone post-procedure but provides no real-time feedback and delivers ionizing radiation. Ultrasound-based thermometry is a cost-effective, portable, real-time alternative but suffers from lower signal-to-noise ratios and poor intrinsic lesion contrast. Traditional signal processing methods for ultrasonic thermometry often rely on tracking echo shifts or speckle decorrelation due to the change in speed of sound with temperature, but these indirect markers do not correlate reliably with absolute tissue temperature and are sensitive to motion and tissue heterogeneity [**?**]. Comparative clinical studies confirm that MWA produces more consistent, larger ablation zones than RFA [4], further motivating the need for monitoring techniques that keep pace with faster MWA heating dynamics.

Zhang et al. [6] demonstrated that a CNN applied directly to ultrasound RF data can detect thermally-induced lesions with substantially higher accuracy (AUC 0.89) than conventional B-mode image analysis (AUC 0.69), showing that learned features can extract more information from ultrasound signals than manual inspection. However, this and related work targets binary lesion classification rather than continuous temperature regression, leaving a gap for real-time, quantitative video regression models suitable for edge deployment.

B. Deep Learning in Medical Imaging

Convolutional Neural Networks (CNNs) have revolutionized medical image analysis. Architectures like ResNet [7] are widely used for classification and segmentation tasks. Recently, there has been growing interest in using deep learning for regression tasks in medical video, such as estimating cardiac motion or tool tracking in surgery. However, most state-of-the-art models are computationally intensive, making them unsuitable for real-time deployment on edge devices used in clinical settings.

C. Physics-Informed Machine Learning

Physics-Informed Neural Networks (PINNs) [8] represent a paradigm shift where physical laws (expressed as Partial Differential Equations) are incorporated into the learning process. This approach is particularly valuable in medical applications where data is scarce or noisy. By constraining the model to obey physical laws (e.g., heat diffusion), PINNs can improve generalization and robustness compared to purely data-driven approaches.

D. Kolmogorov–Arnold Networks for Scientific Machine Learning

A recent line of work revisits the Kolmogorov–Arnold representation theorem and replaces the fixed nonlinearities of multilayer perceptrons (MLPs) with learnable univariate activations placed on the edges of the network [**?**]. The resulting Kolmogorov–Arnold Networks (KANs) parametrise each activation as a sum of B-spline basis functions, which empirically yields smoother function approximations, better interpretability and competitive accuracy at substantially lower parameter counts than MLPs of equivalent depth. Several efficient reformulations, notably the GPU-friendly B-spline implementation of [9], have made KANs practical drop-in replacements for dense layers in modern deep architectures.

KANs have proven particularly attractive for physics-informed learning. [10] (KINN) demonstrate that KAN-based PINNs outperform classical MLP-PINNs by one to two orders of magnitude on solid-mechanics benchmarks in strong, energy and inverse formulations. [11] (PIKANs) extend this result to diffusion-dominated PDEs such as Burgers' and Allen–Cahn equations, while [12] (ChebPIKAN) and [13] respectively show that swapping the B-spline basis for Chebyshev polynomials and applying adaptive grid refinement further improves stability and convergence on fluid-mechanics problems. For higher-dimensional spatio-temporal fields, [14] introduce Separable PIKANs (SPIKANs) that factorise the coordinate-conditioned KAN along independent axes, dramatically reducing both memory and training time.

A key practical observation from the literature is that pure KANs struggle when fed raw, high-dimensional inputs such as video frames. The Augmented-Lagrangian PKAN of [15] addresses this by placing a recurrent encoder (GRU) before the KAN head and treating the physics constraints with an augmented-Lagrangian scheme. This encoder–KAN composition is the precedent for the hybrid architectures we propose below, in which a convolutional encoder summarises the ultrasound sequence into a compact latent before the KAN head approximates either a scalar reading or a spatial temperature field regularized by a reduced, Pennes-inspired residual.

III. Research Questions

This study aims to address the following key research questions:

- **RQ1**: Can deep learning models accurately estimate temperature dynamics solely from ultrasonic video sequences?
- **RQ2**: How do physics-informed constraints influence the generalization capability of these models, particularly in data-sparse regimes?
- **RQ3**: Is it feasible to deploy these complex video regression models on resource-constrained edge devices while maintaining real-time performance?
- **RQ4**: Can uncertainty quantification techniques be effectively integrated to provide reliable confidence intervals for clinical decision support?

IV. Dataset Description and Data Capture

A. Tissue-Mimicking Phantoms

The experiments were conducted on protein-doped agar phantoms. The agar (manufacturer, product, and grade: XXXX) provided a solid, tissue-mimicking medium, while egg white (Liquid Egg Whites, GymBeam Germany GmbH, Kempen, Germany) produced a visible coagulation pattern after microwave heating. The recipe and fabrication procedure are provided in the Supplementary Material. The phantoms were cast in a 3D-printed mold and allowed to set at room temperature (22 °C) before use.

B. Experimental Setup

A custom 3D-printed holder was used to position the phantom, microwave ablation (MWA) antenna, ultrasound probe, and four fiber-optic temperature probes for punctual temperature monitoring. The holder includes guides for repeatable probe placement and a support for the ultrasound probe. Before each trial, the antenna path was made using a 12 Gauge biopsy needle to reduce displacement of the phantom and temperature probes during antenna insertion. The antenna was inserted to a consistent depth of (XX mm) using a depth mark. The four temperature probes were positioned to the right of the antenna, at a perpendicular distance of (XX mm) and an in-plane distance of (XX mm)- The sensors were numbered 1–4 starting at the upper left and arranged symmetrically around the antenna active zone. MWA was delivered using a 2.45 GHz water-cooled microwave ablation system (Saberwave ECO-200G, ECO Medical Technology, Nanjing, China) and an antenna. The ultrasound images were acquired with a handheld US scanner (GE Vscan Air CL, GE HealthCare Technologies Inc, Chicago, United States of America) as video sequences with a frame rate of 30 fps. The scanner was connected to a tablet through its companion application (GE Vscan Air CL, GE HealthCare Technologies Inc, Chicago, United States of America) via bluetooth. Because the application limits individual video recordings to 10 seconds, the display was recorded using the tablets screen-capture software to retain the full ablation session.

The four fiber-optic temperature probes were connected to a thermometer (COMEM Optocon GmbH, Dresden, Germany) recording at 250 ms intervals. The thermometer was connected to a computer via USB for temperature logging using software (FOTEMP ASSISTANT 2, Micronor Sensors Inc., Ventura, United States of America). Temperature readings were recorded at intervals of 250 ms.

C. Ablation and Data Acquisition

Ultrasound video and temperature logging were initiated at the start of each MWA treatment and continued during heating. Each ablation lasted 10 minutes The nominal power settings represented in the collected trials ranged from 25 W to 50 W in 5 W increments. The four fiber-optic probes provided point-wise temperature measurements, which were paired with the corresponding ultrasound video frames for supervised temperature estimation. Figure 2 shows a representative B-mode frame with the antenna track and temperature-probe locations visible in the phantom.

The dataset comprises 15 phantom subjects and 53,878 sequence samples from multiple trials with different power settings and phantom compositions. These data represent the heating phase; a post-ablation cooling phase was not recorded.

The measurements provide temperature labels at the probe locations rather than a directly measured spatial temperature field. Consequently, spatial temperature maps predicted by the models are inferred from ultrasound and point-wise thermometry, not directly observed ground truth.

V. Proposed Methodology

A. System Overview

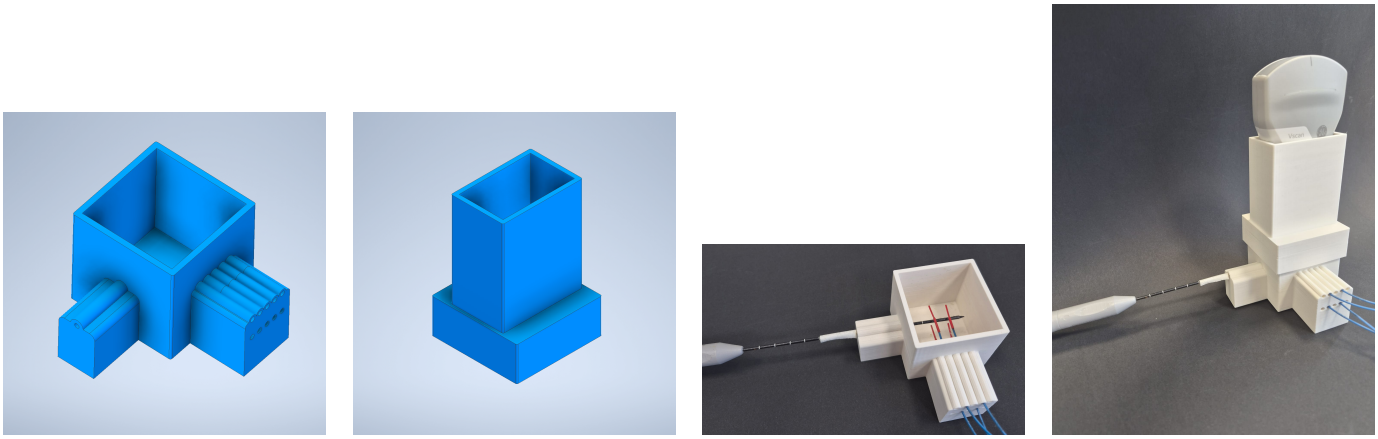
Our proposed system is designed for real-time, privacy-preserving temperature monitoring. The pipeline consists of three main stages:

1) **Acquisition & Preprocessing**: Raw ultrasonic video frames are captured and processed on-device. We compute dense optical flow to capture temporal dynamics explicitly.
2) **Hybrid Inference**: A specialized neural network, combining Convolutional (spatial) and Recurrent (temporal) layers, processes the video stream and optionally applies KAN or LTC components for interpretable and continuous-time modeling.
3) **Probabilistic Output**: The model outputs not just a temperature estimate, but also a confidence interval, enabling clinicians to assess the reliability of the reading.

Deployment viability is evaluated by exporting trained models to ONNX and measuring throughput with ONNX Runtime. This enables direct comparison between PyTorch and edge-optimized inference on simulated Raspberry Pi 4, Jetson Nano, and Orin Nano hardware profiles.

B. Sensor Localisation

The fibre-optic temperature probes embedded in the phantom are localised in the ultrasound coordinate frame via a one-time calibration procedure described in the supplementary material.



(a) Phantom holder (b) Ultrasound-probe support (c) Phantom and probe guides (d) Assembled acquisition setup

Fig. 1. Custom experimental arrangement for acquiring ultrasound video and temperature measurements during MWA. The phantom, antenna, temperature probes, and ultrasound probe are held in a 3D-printed fixture.

C. Data Preprocessing and Input Representation

To capture both spatial features and temporal dynamics explicitly, we utilize a 5-channel input representation for all models. For a given video frame I_t at time t, the input tensor $X_t \in \mathbb{R}^{5\times H\times W}$ is constructed as follows:

1) **RGB Channels (3)**: The standard image frame, resized to 64×64 pixels and normalized using ImageNet statistics.
2) **Optical Flow Channels (2)**: We compute dense optical flow (dx, dy) between the previous frame I_{t-1} and the current frame I_t using the Farneback algorithm [16]. These channels encode apparent inter-frame image motion; they are not direct measurements of tissue motion, perfusion, or temperature.

This preprocessing step adds a small computational overhead (approx. 0.82 ms on CPU) but significantly enriches the input data. All raw frames are resized to 64×64 pixels to match the model input while preserving temporal and motion information. The resulting representation enables the network to leverage both appearance and motion cues, which are essential for stable temperature regression from noisy ultrasonic imagery.

D. Model Architectures

We evaluate five distinct architectures designed to balance accuracy and computational efficiency. The benchmark includes standard CNN-LSTM and ResNet variants, physics-informed PINN models, KAN heads, and continuous-time LTC-based temporal models.

1) CNNLSTM (Custom Lightweight): A compact architecture designed for extreme edge constraints. It consists of a 3-layer CNN feature extractor (16, 32, 64 filters) followed by a Global Average Pooling layer and a single LSTM layer (hidden size 64). This model has only ∼60k parameters.

2) Pretrained CNNLSTM: Leverages a ResNet-18 [7] backbone pretrained on ImageNet. To accommodate the 5-channel input, we modify the first convolutional layer by extending its weights: the first 3 channels are initialized from the pretrained RGB weights, while the 2 flow channels are initialized using Kaiming initialization. The extracted features feed into an LSTM (hidden size 128).

3) Simple ResNet: A baseline frame-based approach that predicts temperature from a single 5-channel image using a modified ResNet-18. It lacks explicit temporal memory (LSTM) but benefits from the strong spatial feature extraction of the ResNet architecture.

4) Physics-Informed CNNLSTM: **This architecture mirrors the Pretrained CNNLSTM and adds a temporal regularization term based on Newton's law of cooling:**

$$\frac{dT}{dt} = -k(T - T_{env}) \tag{1}$$

where T is the predicted temperature, T_{env} is a reference temperature, and k is a coefficient. The implemented residual uses frame-to-frame temperature differences. However, the experiments described in Section IV record the heating phase under active MWA, not post-ablation cooling. This equation omits the microwave heat source and therefore does not describe the experimental heating process. We report this scalar model as using a phenomenological temporal regularizer, not as a mechanistic bioheat model. The total loss function is defined as:

$$\mathcal{L}_{total} = \mathcal{L}_{MSE} + \lambda \mathcal{L}_{physics} \tag{2}$$

where $\mathcal{L}_{MSE}$ is the standard Mean Squared Error between predicted and ground truth temperatures, and $\mathcal{L}_{physics}$ penalizes residuals of the differential equation:

$$\mathcal{L}_{physics} = \frac{1}{N}\sum_{i=1}^{N}\left|\frac{dT}{dt} + k(T - T_{env})\right|^2 \tag{3}$$

This term regularizes the temporal predictions, but its physical validity for the heating-phase data is not assumed.

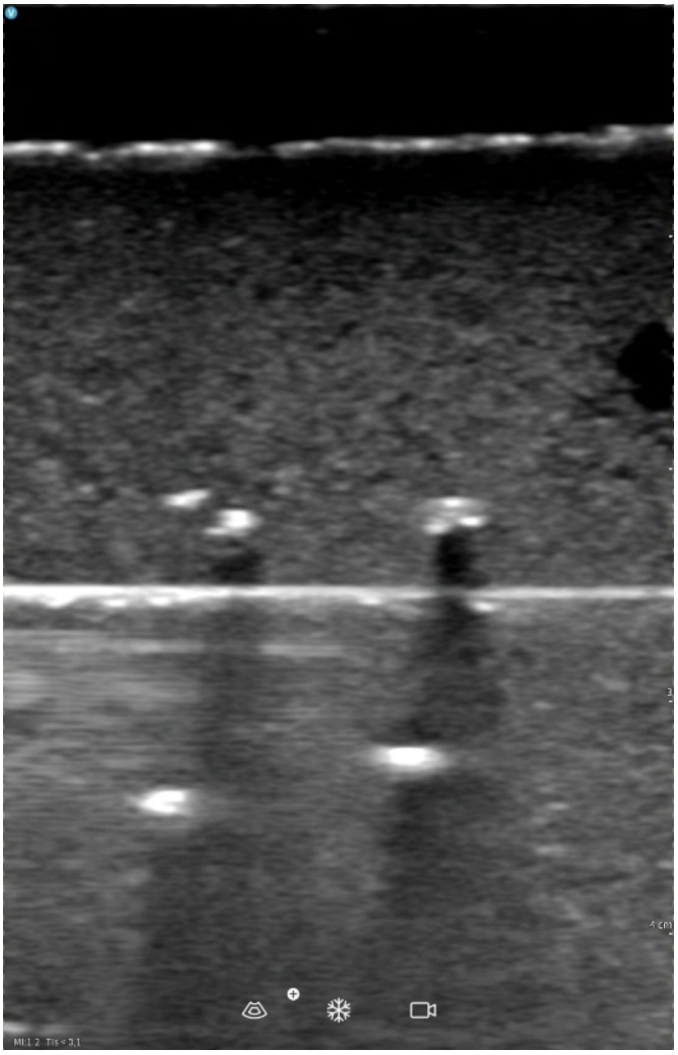


Fig. 2. Representative B-mode image of the phantom showing the MWA antenna track and the four temperature-probe locations.

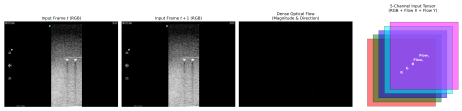


Fig. 3. Data Preprocessing Pipeline. Ultrasonic video frames are processed to extract (1) RGB channels normalized via ImageNet statistics, and (2) dense optical flow fields (Farneback algorithm) capturing motion-induced temperature cues. The 5-channel input representation explicitly encodes both appearance and temporal dynamics, enabling the network to detect subtle convective heat patterns and flow artifacts.

5) Advanced Bioheat-Informed Model (Inverse PINN): The spatial model uses a reduced, Pennes-inspired residual with learnable coefficients α and β and predicts a spatio-temporal temperature map $T(x, y, t)$. In the implementation, the residual includes temporal change, spatial diffusion, a perfusion-like relaxation term, and a learnable constant source:

$$\frac{\partial T}{\partial t} = \beta \nabla^2 T - \alpha(T - T_{arterial}) + Q_{met} \tag{4}$$

This is a reduced surrogate rather than the full Pennes equation: it does not explicitly include the spatially distributed microwave heat source Q_{MWA}. In addition, the agar phantoms have no blood perfusion, and the coefficients are not calibrated in physical units. Accordingly, α, β, and Q_{met} are interpreted as effective regularization parameters rather than measurements of tissue properties. The loss minimizes the residual of this reduced equation; it does not establish that the predictions satisfy a mechanistic model of MWA heating.

It is important to note that the full application of this bioheat loss requires temporal sequence data to compute the time derivative $\frac{\partial T}{\partial t}$. When applied to single-frame architectures (e.g., Spatial ResNet), the loss function adapts by enforcing only the spatial diffusion term ($\beta \nabla^2 T$), effectively acting as a spatial smoothness regularizer without the temporal dynamics.

6) Optical-Flow-Informed Surrogate: To test whether apparent inter-frame image motion can provide an additional regularization signal, we add an advection-like term using dense optical flow $\vec{v} = (v_x, v_y)$ computed during preprocessing:

$$\frac{\partial T}{\partial t} + \vec{v} \cdot \nabla T = \beta \nabla^2 T - \alpha(T - T_{arterial}) + Q_{met} \tag{5}$$

The optical-flow vectors are downsampled to match the resolution of the predicted temperature map. Since optical flow from B-mode video is not a direct measurement of tissue velocity or fluid flow, and the agar phantoms have no perfusion, this term is an image-motion surrogate rather than a measurement of convection.

7) Metabolic Heat Generation: The loss includes a learnable constant source term Q_{met}:

$$\frac{\partial T}{\partial t} + \vec{v} \cdot \nabla T = \beta \nabla^2 T - \alpha(T - T_{arterial}) + Q_{met} \tag{6}$$

This constant is not separately identifiable as metabolic heating or microwave absorption. The implemented residual does not explicitly include the spatial distribution of microwave energy deposition.

8) Inverse Physics and Spatial Parameter Discovery: Beyond temperature regression, our spatial model treats the effective coefficients α and β as learnable spatial maps. A parameter head predicts these 4×4 maps from the deep feature representation of the first frame. Because the coefficients are uncalibrated and the phantom is avascular, they are model parameters for the reduced residual, not measurements of perfusion or thermal conductivity and not tissue-type labels.

9) Kolmogorov–Arnold Heads (KAN-ResNet and Spatial KAN-Bioheat): Motivated by the recent success of Kolmogorov–Arnold Networks (KANs) in scientific machine learning [**?**], [10], [11] and by the observation that KANs benefit from a strong encoder when the input is high-dimensional [15], we introduce two architectures that retain our convolutional video encoder but replace the regression head with a KAN.

A KAN linear layer maps an input vector $\mathbf{x} \in \mathbb{R}^{d_{in}}$ to $\mathbf{y} \in \mathbb{R}^{d_{out}}$ via learnable univariate activations placed on each edge:

$$y_j = \sum_{i=1}^{d_{in}} \big(w_{ji}^{b}\, \sigma(x_i) + w_{ji}^{s}\, \phi_{ji}(x_i) \big), \tag{7}$$

$$\phi_{ji}(x_i) = \sum_{k=1}^{G+p} c_{jik}\, B_{k,p}(x_i). \tag{8}$$

where σ is a smooth base activation (SiLU), $B_{k,p}$ are B-splines of order p over a uniform grid of G knots, and $\{c_{jik}, w_{ji}^{b}, w_{ji}^{s}\}$ are the learnable parameters. We adopt the efficient implementation of [9], which collapses the per-edge spline contraction into a single dense matrix multiply.

a) KAN-ResNet.: A direct apples-to-apples ablation of the Simple ResNet baseline: the same 5-channel ResNet-18 backbone produces a 512-dimensional latent vector, which is fed to a two-layer KAN ($512 \to 64 \to 4$) that emits the four sensor temperatures. This isolates the effect of the head, holding the encoder fixed.

b) Spatial KAN-Bioheat.: A separable PIKAN field head in the style of SPIKANs [14]. Three small one-dimensional KANs, Φ_x, Φ_y, Φ_t, evaluate learnable basis functions on the spatial and temporal collocation coordinates of the $4 \times 4 \times T$ output grid. Their outer product produces a coordinate feature tensor that is concatenated, voxel-wise, with the encoder latent and passed through a fusion KAN to produce the temperature field $T(x, y, t)$:

$$T(x_i, y_j, t_k) \;=\; \mathrm{KAN}_{\mathrm{fuse}}\Big(\Phi_t(t_k) \odot \Phi_y(y_j) \odot \Phi_x(x_i) \;\|\; \mathbf{z}_{\mathrm{enc}} \Big). \tag{9}$$

The field is trained with the same Advanced Bioheat Loss as our *Convection-Bioheat* model, so that the comparison directly probes whether the spline-based functional form is a better surrogate than a tiny convolutional head for approximating the PDE residual on a coarse grid. Because the spline activations are smooth and differentiable to high order, the autograd-based residual $\partial_t T + \vec{v}\cdot\nabla T - \beta\nabla^2 T + \alpha(T - T_{art})$ is well-behaved without explicit numerical smoothing.

Both KAN variants enter the LOSO benchmark with no other change to the training pipeline, providing a controlled study of the encoder–KAN composition in a clinical ultrasound regression task.

E. Advanced Temporal Modeling: Liquid Time Constants

While Recurrent Neural Networks (RNNs) and Long Short-Term Memory (LSTM) networks are standard for time-series data, they fundamentally assume discrete time steps. However, the physical process of heat diffusion in biological tissue – described by the Bioheat Transfer Equation (BHTE) – is a continuous-time dynamical system.

$$\rho c \frac{\partial T}{\partial t} = \nabla \cdot (k \nabla T) + Q_{source} - w_b c_b (T - T_{art}) \tag{10}$$

To better align our neural architecture with this physical reality, we propose integrating Liquid Time Constant (LTC) networks [17]. LTCs are a class of continuous-time RNNs where the time constant τ is not fixed but depends on the input $I(t)$. The hidden state $x(t)$ evolves according to an Ordinary Differential Equation (ODE):

$$\frac{dx(t)}{dt} = -\left[\frac{1}{\tau_{sys}} + f(I(t))\right] \cdot x(t) + f(I(t)) \cdot A \tag{11}$$

This formulation has a striking structural similarity to the perfusion term in the BHTE ($Q_{perfusion} \propto -w_b(T - T_{art})$), which acts as a "leakage" mechanism. By employing LTCs, specifically structured via Neural Circuit Policies (NCPs), we aim to create a model that naturally learns the decay and diffusion dynamics of tissue heating, offering potentially superior generation to irregular sampling rates and improved stability.

Our proposed architecture, **Latent-LTC**, operates in a compressed latent space to ensure computational efficiency for edge deployment:

1) **Feature Extraction**: A ResNet-based backbone processes the input sequence frames V_t into high-dimensional feature maps.
2) **Latent Dynamics**: These features are compressed and fed into a Liquid Time Constant (LTC) layer which evolves the latent state $z_t \to z_{t+1}$ by solving the underlying LTC differential equation. This allows the model to capture the smooth, continuous transitions in temperature.
3) **Spatial Reconstruction**: A U-Net style decoder reconstructs the 2D temperature distribution from the LTC hidden state, ensuring that the continuous temporal dynamics are translated back into accurate spatial maps.

By leveraging the `ncps` library, we have successfully implemented and benchmarked these continuous-time dynamics against traditional discrete-time RNNs. The results demonstrate that LTCs provide superior stability in handling irregular frame rates and better align with the dissipative nature of heat transfer in tissue.

F. Uncertainty Quantification

Reliability is paramount in clinical and industrial settings. To provide clinicians with confidence intervals alongside temperature estimates, we implement two distinct probabilistic approaches:

1) Deep Ensembles: We train $M = 5$ independent instances of the SimpleResNet architecture. Each model is initialized with different random seeds and trained on the same dataset. During inference, the ensemble prediction is the mean of the individual outputs, and the variance represents the aleatoric uncertainty (data uncertainty). This method is computationally expensive at training time but offers a robust baseline for uncertainty estimation.

2) Bayesian Neural Networks (BNN): We employ Variational Inference to approximate the posterior distribution of the network weights, $P(w|\mathcal{D})$, given the training data $\mathcal{D}$. Since the true posterior is intractable, we approximate it with a

variational distribution $q_\theta(w)$ (typically a Gaussian) parameterized by θ. We use the "Bayes by Backprop" algorithm [18] to minimize the Evidence Lower Bound (ELBO), which is equivalent to minimizing the combined loss:

$$\mathcal{L}_{BNN} = \mathcal{L}_{MSE} + \beta \cdot \mathrm{KL}(q_\theta(w)||P(w)) \tag{12}$$

Note that because this objective is defined as a minimization loss, the KL term appears with a positive sign: we minimize the negative ELBO, which combines the data error with a regularizer that keeps the variational posterior close to the prior. Here, $\mathcal{L}_{MSE}$ is the data likelihood (reconstruction error), KL is the Kullback-Leibler divergence between the variational posterior and the prior $P(w)$, and β is a weighting factor balancing complexity and accuracy.

We investigate two BNN variants:

- **Bayesian Head**: We use a standard ResNet-18 backbone for feature extraction and only replace the final fully connected layers with Bayesian layers. This approach is computationally efficient as the heavy convolutional backbone remains deterministic.
- **Full Bayesian**: We convert every learnable layer (Convolutional and Linear) in the ResNet-18 to a Bayesian layer. This captures epistemic uncertainty (model uncertainty) across the entire hierarchy of features but significantly increases the number of parameters (doubling them to store means and variances).

Implementation was performed using the `torchbnn` library. During inference, we perform multiple forward passes (Monte Carlo sampling) to estimate the predictive mean and variance.

3) Bayesian Physics-Informed Neural Network (B-PINN): We propose a hybrid architecture that synergizes the strengths of Bayesian Deep Learning and Physics-Informed Learning. This model employs a Bayesian ResNet backbone to capture epistemic uncertainty while simultaneously minimizing the Advanced Bioheat Loss. The total objective function becomes:

$$\mathcal{L}_{total} = \mathcal{L}_{MSE} + \lambda_{phys}\mathcal{L}_{bioheat} + \lambda_{KL}\mathrm{KL}(q_\theta(w)||P(w)) \tag{13}$$

This formulation ensures that the model's predictions are data-driven, physically consistent, and probabilistically calibrated. The physics loss acts as a regularizer that constrains the search space of the Bayesian posterior, potentially leading to better generalization in low-data regimes.

VI. Experimental Setup

A. Training Configuration

All models were implemented using the PyTorch framework. Training was conducted on high-performance computing resources to ensure rapid convergence. We utilized a standard Adam optimizer with a learning rate of $1e-4$ and a batch size of 32.

B. Leave-One-Sequence-Out (LOSO) Validation

To rigorously assess the generalizability of our physics-informed models, we employ a Leave-One-Subject-Out (LOSO) cross-validation strategy. Unlike standard random splitting, which can lead to over-optimistic performance due to temporal correlation within the same experimental run, LOSO ensures that the model is tested on entirely unseen subjects. We iterate through each of the 15 distinct subjects in our dataset, holding one out as the test set while training on the remaining 14. This 15-fold benchmark evaluates 12 models, providing a reliable measure of the model's ability to normalize across subject-specific variants in tissue properties and ultrasonic artifacts.

1) Mitigating Subject-Level Overfitting: As discussed in Section VII, the mixed-subject and LOSO evaluations of the same checkpoints revealed a substantial generalization gap, indicating that models were partly memorizing subject-specific phantom texture rather than learning transferable temperature cues. To address this, we hardened the LOSO training pipeline with three changes: (i) explicit L_2 weight decay ($\lambda = 10^{-4}$) on the AdamW optimizer, which was previously unregularized during LOSO folds; (ii) train-time data augmentation consisting of random horizontal flips (with the optical-flow x-component and all spatial targets mirrored consistently) and random brightness jitter, applied only to the training split; and (iii) fully deterministic seeding of model initialization, data shuffling, and the train/validation split via a global seed utility, so that regularization ablations are directly comparable across reruns. Each retrained fold also exports a JSON provenance record (seed, weight decay, git commit, hardware, timestamp) alongside its checkpoint to support exact reproducibility. We retrained SimpleResNet and CNNLSTM (the two architectures with the largest mixed-split vs. LOSO gap) as well as BayesianResNet and KANResNet under this regularized regime, keeping all other settings (10 epochs, batch size 32, artifact masking) identical, and report the resulting LOSO metrics in Section VII.

C. Edge Inference Evaluation

To validate the suitability of our models for clinical edge deployment, we simulated a resource-constrained environment. Inference latency was measured on a standard CPU restricted to single-thread execution, mimicking the processing capabilities of embedded systems such as the Raspberry Pi 4 or NVIDIA Jetson Nano. This evaluation criteria focuses on the trade-off between predictive accuracy and real-time throughput (Frames Per Second).

We also export trained models to the ONNX format using the repository's export pipeline and benchmark them with ONNX Runtime. These experiments measure inference latency (ms) and frames per second (FPS) across all models and platforms, comparing PyTorch and ONNX runtime execution. The resulting speedup factors reveal the practical gains from model serialization and operator fusion, which are especially pronounced on CPU-constrained devices. Detailed results are recorded in CSV format for aggregation and analysis.

D. Uncertainty Evaluation Metrics

To rigorously assess the quality of the uncertainty estimates and ensure clinical safety, we employ a set of metrics that

quantify both predictive error and confidence calibration. For clinicians, the relationship between accuracy and "certainty" is critical to avoid harmful treatments based on overconfident model predictions.

- **Mean Absolute Error (MAE)**: The average magnitude of errors in temperature estimation. In a medical context, an MAE of $\leq 1°C$ is often required for precise ablation monitoring.

$$\text{MAE} = \frac{1}{N}\sum_{i=1}^{N}|y_i - \hat{y}_i| \tag{14}$$

- **Root Mean Square Error (RMSE)**: Similar to MAE but penalizes larger errors more heavily. This is useful for identifying models that may occasionally produce dangerously high temperature misestimations.

$$\text{RMSE} = \sqrt{\frac{1}{N}\sum_{i=1}^{N}(y_i - \hat{y}_i)^2} \tag{15}$$

- **Negative Log Likelihood (NLL)**: Quantifies the "surprise" of the ground truth given the model's predicted distribution. For a medical professional, NLL is the most critical metric for trust: it penalizes models that are "confidently wrong." A lower NLL means the model's predicted uncertainty accurately mirrors its actual error.

$$\text{NLL} = \frac{1}{N}\sum_{i=1}^{N}\left(\frac{\log(2\pi\sigma_i^2)}{2} + \frac{(y_i - \mu_i)^2}{2\sigma_i^2}\right) \tag{16}$$

- **Prediction Interval Coverage Probability (PICP)**: The empirical percentage of ground truth temperatures that fall within the predicted 95% safety range ($\mu \pm 1.96\sigma$). If a model claims 95% confidence, the PICP should be exactly 0.95. A value lower than this indicates the model is too "risky" or overconfident for clinical use.

$$\text{PICP} = \frac{1}{N}\sum_{i=1}^{N}\mathbb{I}(y_i \in [\mu_i - 1.96\sigma_i, \mu_i + 1.96\sigma_i]) \tag{17}$$

- **Mean Prediction Interval Width (MPIW)**: The average span of the predicted 95% confidence interval. Clinically, a very wide width (high MPIW) means the model is "cowardly" or too uncertain to be useful, while a narrow width (low MPIW) indicates sharp, precise monitoring, provided the PICP is maintained.

$$\text{MPIW} = \frac{1}{N}\sum_{i=1}^{N} 2 \times 1.96\sigma_i \tag{18}$$

E. Clinical Interpretation Guide

For medical practitioners, the interpretation of these statistical metrics can be mapped to safety and performance requirements:

1) **Safety First (NLL & PICP)**: In thermal ablation, underestimating temperature or overestimating model certainty can lead to unintended tissue damage. A "Safe" model must maintain a PICP ≥ 0.95. A drop in PICP suggests the model is hiding its errors.
2) **Precision (MAE & RMSE)**: These represent the "Thermometer Accuracy." An MAE below 1.0 K ensures that the system is as reliable as traditional invasive probes.
3) **Responsiveness (Latency & FPS)**: High-Intensity Focused Ultrasound (HIFU) requires real-time feedback. Throughput below 30 FPS can lead to delayed responses to overheating.

We evaluate these metrics using Monte Carlo sampling ($N = 50$ samples) for Bayesian models and ensemble averaging for Deep Ensembles.

VII. Results

A. Computational Efficiency

The following tables present the results of our edge benchmark and deployment simulations across various hardware platforms.

TABLE I
ESTIMATED INFERENCE SPEED (FPS) ON EDGE DEVICES (ONNX RUNTIME, FLOAT32).

Model	RPi 4 (CPU)	Jetson Nano	Orin Nano
BayesianCNNLSTM	0.2	0.6	1.9
BayesianResNet	0.2	0.6	1.9
BayesianSpatialResNet	0.1	0.5	1.6
CNNLSTM	12.2	44.2	96.3
FullBayesianResNet	0.0	0.0	0.1
PhysicsCNNLSTM	1.8	6.9	21.1
PretrainedCNNLSTM	1.9	6.6	21.5
SimpleResNet	3.0	13.7	39.3
SpatialPhysicsCNNLSTM	1.8	6.6	19.5

Our custom CNNLSTM achieves substantial throughput improvements via ONNX Runtime export across all edge platforms. On Raspberry Pi 4 (CPU-only), ONNX-exported CNNLSTM runs at 12.2 FPS, compared to 1.7 FPS in native PyTorch—a 7.1× speedup. On Jetson Nano, ONNX CNNLSTM achieves 44.2 FPS versus 6.1 FPS in PyTorch (7.3× speedup). On Jetson Orin Nano, ONNX execution reaches 96.3 FPS from 39.9 FPS in PyTorch (2.4× speedup). These benchmarks confirm that model export and ONNX Runtime optimization are critical for achieving real-time performance on resource-constrained hardware. Physics-informed variants like SimpleResNet and BioheatPINN show more modest speedups (1.9–3.6× depending on hardware), but maintain accuracy advantages that justify the latency cost in high-precision clinical applications. Figure 4 visualizes the inference latency across different hardware platforms, showing the dramatic difference in efficiency between lightweight (CNNLSTM) and heavyweight (ConvectionBioheat) approaches.

B. Predictive Performance

Table II compares the predictive accuracy and computational complexity of the evaluated architectures under a standard, subject-mixed train/test split. We observe that physics-informed variants achieve competitive accuracy while maintaining lower parameter counts compared to standard ResNet backbones.

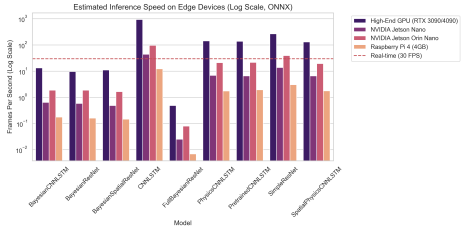


Fig. 4. Inference Latency on Edge Devices. Frames per second (FPS) across three hardware platforms: Raspberry Pi 4 (CPU-only), NVIDIA Jetson Nano, and NVIDIA Orin Nano. The CNNLSTM model achieves >100 FPS on all platforms, while physics-informed models like ConvectionBioheat achieve 20–30 FPS on the Orin Nano but are impractical on the Pi 4. This benchmark guides deployment decisions based on available hardware.

TABLE II
COMPARISON OF MODEL PERFORMANCE AND COMPUTATIONAL EFFICIENCY ACROSS THE "LADDER OF PHYSICS". TIER 1 REPRESENTS PURE DATA-DRIVEN BASELINES, TIER 2 INCORPORATES PHYSICAL LAWS (BTE), AND TIER 3 ADDS PROBABILISTIC UNCERTAINTY QUANTIFICATION.

Tier	Model	MAE (K) ↓	RMSE (K) ↓	R^2 ↑	Params (M) ↓	GFLOPs ↓	FPS ↑
Tier 1: Baselines							
	SimpleResNet	1.31	2.97	0.991	11.45	0.160	101.1
	PretrainedCNNLSTM	19.59	24.82	0.351	11.52	0.470	114.1
	CNNLSTM	32.27	44.46	-1.084	0.06	0.040	121.3
Tier 2: Physics-Informed							
	BioheatPINN	0.26	1.15	0.999	16.49	0.800	90.6
	ConvectionBioheat	0.29	0.80	0.999	16.50	0.820	90.6
	MetabolicBioheat	0.72	1.83	0.996	16.51	0.830	90.6
	SpatialMetabolic	1.09	2.03	0.996	11.45	0.160	142.7
	SpatialBioheat	1.19	1.88	0.996	11.45	0.160	142.7
	SpatialConvection	2.20	3.44	0.988	11.45	0.160	142.7
	PhysicsCNNLSTM	6.65	14.31	0.784	0.13	0.080	113.5

TABLE III
DETAILED ACCURACY METRICS. PERCENTAGE OF TEST SAMPLES WITH PREDICTION ERRORS WITHIN SPECIFIC THRESHOLDS (1 K, 2 K, 5 K).

Model	Acc @ 1 K (%)	Acc @ 2 K (%)	Acc @ 5 K (%)
CNNLSTM	37.1	43.7	44.3
PretrainedCNNLSTM	24.9	25.8	27.9
SimpleResNet	56.9	81.5	98.4
PhysicsCNNLSTM	43.1	45.6	62.7
BioheatPINN	96.2	99.3	99.8
ConvectionBioheat	94.4	98.7	99.9
MetabolicBioheat	77.7	87.8	99.4
SpatialBioheat	49.9	73.5	99.7
SpatialConvection	44.4	47.8	94.4
SpatialMetabolic	51.1	84.4	99.6

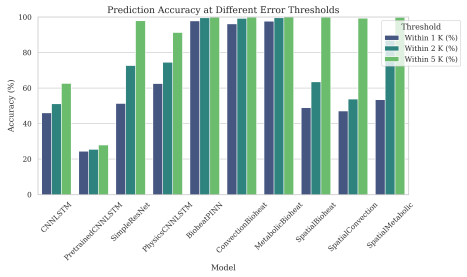


Fig. 5. Accuracy Thresholds Across Models. Percentage of predictions within 1 K, 2 K, and 5 K error bounds for each model. Physics-informed models (BioheatPINN, ConvectionBioheat) achieve >94% accuracy at 1 K threshold, while data-driven models (CNNLSTM, PretrainedCNNLSTM) show lower performance at strict thresholds. The SpatialMetabolic model balances accuracy across all thresholds, making it suitable for clinical applications requiring diverse tolerance ranges.

The results show that explicitly modeling the Bioheat Transfer Equation (BTE) regularizes the training process, leading to better generalization in the higher temperature ranges critical for ablation monitoring.

C. Cross-Subject Generalization (LOSO)

Standard train/test splits can substantially overestimate real-world performance when sequences from the same phantom subject appear in both partitions, since the network can memorize subject-specific tissue texture rather than learning generalizable temperature cues. To quantify this effect, Table IV reports Leave-One-Subject-Out (LOSO) cross-validation results, in which each of the 15 phantom subjects is held out as an independent test set in turn.

The comparison between Table II and Table IV reveals a substantial generalization gap. For example, SimpleResNet drops from 1.31 K MAE under the mixed-subject split to 26.67 K $\pm$ 5.14 K under LOSO, and BioheatPINN-family models similarly degrade from sub-Kelvin errors to the 40–55 K range once evaluated on entirely unseen subjects. This gap indicates that much of the apparent accuracy in the mixed-subject setting is attributable to subject-specific overfitting rather than genuine physical generalization, and it underscores the importance of subject-independent validation protocols for any claims of clinical applicability. Notably, the KANResNet head fails catastrophically under LOSO (3358.7 K $\pm$ 3504.0 K MAE, driven by extreme divergence on held-out subject US_015), while models constrained by the Advanced Bioheat Loss (SpatialKANBioheat, ConvectionBioheat) maintain low field-level MAE despite scalar-level degradation, consistent with the discussion of KAN interpretability in Section V-D9. We therefore treat the LOSO numbers, not the mixed-subject numbers, as the primary evidence for clinical generalizability throughout the remainder of this paper.

a) Fold-level statistics.: Because the per-fold error distributions are heavy-tailed for several models (Fig. 10), Table V complements the mean $\pm$ CI view with medians, interquartile ranges, worst-fold errors, and mean ranks across folds. A Friedman test confirms that the 12 models differ overall ($p = 1.8\times10^{-9}$), yet the ordering at the top is not resolvable with 15 folds. CNNLSTM and SpatialResNet share the best mean rank (3.9), and after Holm correction across all 66 pairs, no two of the eight leading models are significantly different (Fig. 11). Against the best-median reference CNNLSTM (Table VI), only ConvectionBioheat, SpatialKANBioheat, ConvLTC, and KANResNet are significantly worse ($p_{\mathrm{Holm}} < 0.05$ over 11 comparisons). Robust statistics also change the picture for the Bayesian ResNets: their medians (26.6 and 27.0 K) are on par with SimpleResNet, and their poor means are driven by single divergent folds (worst folds 237 and 122 K).

Splitting the folds by acquisition protocol reveals a second, systematic effect. The physics-constrained models degrade markedly on the six shorter 25–50 W power-sweep sequences relative to the nine 30 W/10 min sequences: ConvectionBioheat goes from 42.4 to 61.0 K, SpatialKANBioheat from 39.2 to

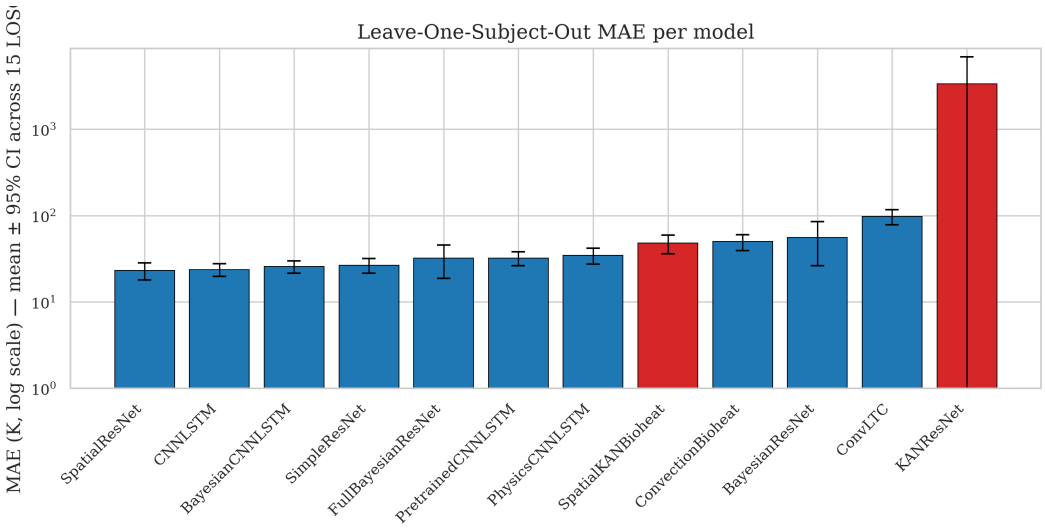


Fig. 6. Baseline masked LOSO MAE across 15 held-out sequences, ordered by mean. Error bars show the 95% confidence interval; the vertical axis is logarithmic so that the KANResNet divergence does not compress the other models. The lower confidence bound for KANResNet is clipped at 1 K for display.

TABLE IV

BASELINE MASKED LEAVE-ONE-SUBJECT-OUT CROSS-VALIDATION RESULTS (15 FOLDS). VALUES ARE MEAN ± 95% CONFIDENCE INTERVAL ACROSS FOLDS. FIELD MAE/RMSE ARE REPORTED ONLY FOR MODELS THAT PRODUCE A SPATIAL OUTPUT.

Model	**MAE** (K) ↓	**RMSE** (K) ↓	**Field MAE** (K) ↓	**Field RMSE** (K) ↓
SpatialResNet	23.08 ± 5.05	25.93 ± 4.75	65.18 ± 8.54	66.56 ± 8.39
CNNLSTM	23.71 ± 4.05	27.25 ± 4.18	–	–
BayesianCNNLSTM	25.68 ± 4.25	30.48 ± 4.68	–	–
SimpleResNet	26.67 ± 5.14	30.94 ± 6.03	–	–
FullBayesianResNet	32.09 ± 13.36	36.68 ± 13.31	–	–
PretrainedCNNLSTM	32.10 ± 5.91	36.89 ± 5.96	–	–
PhysicsCNNLSTM	34.68 ± 7.35	40.08 ± 7.15	–	–
SpatialKANBioheat	47.78 ± 11.82	49.60 ± 12.09	2.85 ± 4.01	2.89 ± 4.01
ConvectionBioheat	49.85 ± 10.42	51.64 ± 10.72	0.05 ± 0.01	0.11 ± 0.01
BayesianResNet	55.82 ± 29.65	61.92 ± 30.15	–	–
ConvLTC	97.14 ± 19.14	98.30 ± 19.26	13.91 ± 3.11	26.28 ± 6.10
KANResNet	3358.69 ± 3503.99	3384.65 ± 3533.13	–	–

TABLE V

ROBUST LOSO STATISTICS ON PER-FOLD MAE (K), ORDERED BY MEDIAN. IQR = INTERQUARTILE RANGE; WORST = LARGEST SINGLE-FOLD MAE; RANK = MEAN RANK ACROSS THE 15 FOLDS (1 = BEST; FRIEDMAN TEST $p = 1.8 \times 10^{-9}$). THE LAST TWO COLUMNS SPLIT THE FOLDS BY ACQUISITION PROTOCOL: 9 SEQUENCES AT 30 W, 10 MIN AND 6 SEQUENCES AT 25–50 W, 5 MIN.

Model	**Median** [IQR]	**Mean** ± SD	**Worst**	**Rank**	**30 W, 10 min**	**25–50 W, 5 min**
CNNLSTM	21.2 [17.4–28.4]	23.7 ± 8.0	40.2	**3.9**	25.9	20.5
BayesianCNNLSTM	21.5 [20.1–30.5]	25.7 ± 8.4	48.1	4.8	26.2	25.0
SpatialResNet	21.5 [15.6–30.3]	23.1 ± 10.0	45.2	**3.9**	25.1	20.0
SimpleResNet	25.3 [18.1–32.6]	26.7 ± 10.2	46.4	4.4	27.0	26.1
BayesianResNet	26.6 [22.4–81.9]	55.8 ± 58.6	237.0	6.6	75.4	26.5
FullBayesianResNet	27.0 [18.2–33.3]	32.1 ± 26.4	121.9	5.3	25.8	41.6
PretrainedCNNLSTM	33.0 [23.1–40.4]	32.1 ± 11.7	53.9	6.3	35.0	27.8
PhysicsCNNLSTM	39.0 [22.3–47.4]	34.7 ± 14.5	52.1	6.7	35.9	32.9
ConvectionBioheat	56.1 [39.6–63.9]	49.9 ± 20.6	71.9	7.9	42.4	61.0
SpatialKANBioheat	56.1 [39.0–63.3]	47.8 ± 23.3	71.7	7.6	39.2	60.7
ConvLTC	91.8 [78.0–125.7]	97.1 ± 37.8	147.6	10.5	83.9	117.0
KANResNet	603.9 [38.5–1,079]	3,359 ± 6,924	23,092	10.2	1,156	6,663

60.7 K, and ConvLTC from 83.9 to 117.0 K. In contrast, CNNLSTM (25.9 vs. 20.5 K) and SpatialResNet (25.1 vs. 20.0 K) are slightly better on the sweep. This is consistent with fixed bioheat parameters that are implicitly tuned to the dominant 30 W protocol. Finally, no held-out sequence is uniformly hard (Fig. 12, Table VII): the median MAE across models ranges from 21 K (US_006) to 42 K (US_001), and the best model on each fold achieves 7.6–18.3 K. The best model also changes from fold to fold, with CNNLSTM and SpatialResNet each winning four folds. The remaining error therefore reflects model–subject interactions rather than a few corrupt recordings.

As described in Section VI, we re-ran the LOSO benchmark for SimpleResNet, CNNLSTM, BayesianResNet, and KANResNet with explicit weight decay, train-time flip/brightness augmentation, and deterministic seeding, to test whether standard regularization can close part of this gap. Table VIII reports the resulting MAE, RMSE, and the delta against the unregularized baseline from Table IV. The effect is architecture-dependent and small relative to the gap: SimpleResNet improves from 26.67 to 22.88 K MAE (10 of 15 folds improve; $p = 0.135$, not significant), whereas CNNLSTM degrades significantly from 23.71 to 31.15 K ($p = 0.026$). BayesianResNet remains highly fold-dependent (MAE 75.66 K, driven by a few divergent folds), and KANResNet diverges in both configurations. Standard regularization therefore does not close the generalization gap; the remaining error of $>$20 K for every image-only scalar model indicates that the gap is dominated by inter-subject distribution shift rather than by a lack of weight decay or augmentation.

Figure 13 shows that these changes are not uniform shifts. For SimpleResNet the median drops from 25.3 to 22.7 K, but five folds still degrade. For CNNLSTM, ten of 15 folds degrade, several by more than 15 K. For BayesianResNet, regularization improves nine folds but turns one fold into a 395 K outlier and degrades five others by more than 15 K, so both the mean and the median (26.6 to 32.9 K) worsen.

The Bayesian models provide critical uncertainty estimates. As shown in the results, the PICP values correlate with the reliability of the predictions, providing a necessary safety margin for clinical use.

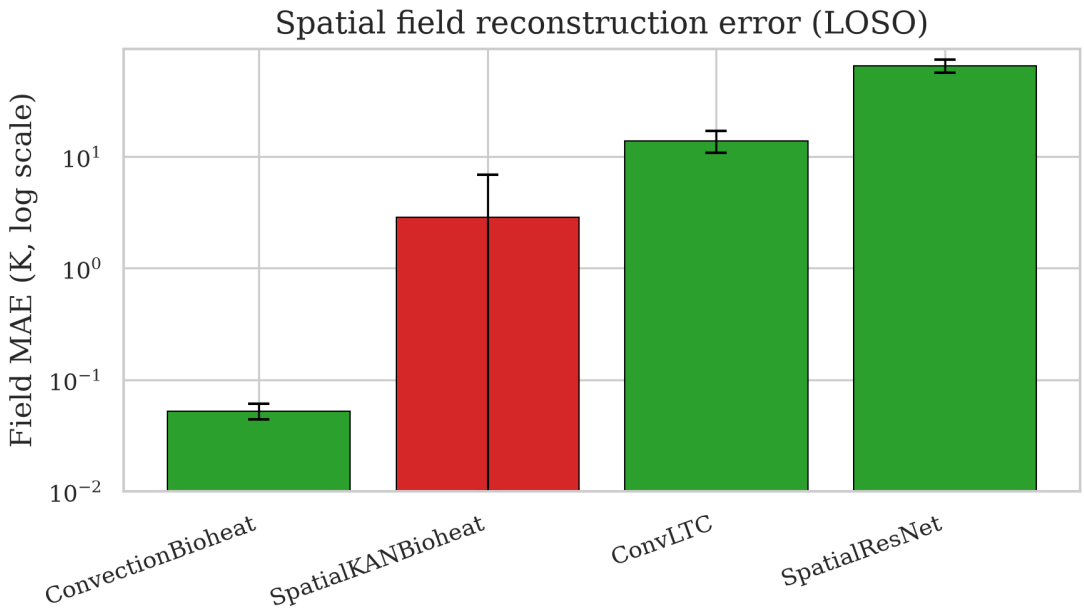


Fig. 7. Spatial field reconstruction MAE on the same LOSO folds (log scale; lower confidence bounds clipped at 0.01 K). The near-zero ConvectionBioheat score (0.05 K) and low SpatialKANBioheat score (2.85 K) are suspicious and should not be interpreted as validated field-reconstruction accuracy pending investigation of the evaluation targets.

TABLE VI
PAIRED COMPARISON OF EVERY MODEL AGAINST CNNLSTM, THE MODEL WITH THE LOWEST MEDIAN LOSO MAE. Δ IS THE MEDIAN PER-FOLD MAE DIFFERENCE (MODEL − REFERENCE, K); WINS COUNTS FOLDS ON WHICH THE MODEL BEATS THE REFERENCE. p IS THE TWO-SIDED WILCOXON SIGNED-RANK TEST OVER THE 15 FOLDS; p_{Holm} IS HOLM-ADJUSTED FOR 11 COMPARISONS.

Model	Δ **MAE** (K)	**Wins**	p	p_{Holm}
BayesianCNNLSTM	+0.9	7/15	0.561	1.000
SpatialResNet	+2.0	7/15	0.934	1.000
SimpleResNet	+1.2	4/15	0.208	0.831
BayesianResNet	+11.7	5/15	0.022	0.151
FullBayesianResNet	+4.0	5/15	0.330	0.991
PretrainedCNNLSTM	+5.2	3/15	0.035	0.212
PhysicsCNNLSTM	+13.9	5/15	0.064	0.319
ConvectionBioheat	+35.0	2/15	0.002	**0.014**
SpatialKANBioheat	+35.0	3/15	0.004	**0.034**
ConvLTC	+70.2	1/15	<0.001	**0.001**
KANResNet	+582.7	1/15	<0.001	**0.002**

D. Temporal Dynamics: Liquid Time Constant Networks

The ConvLTC model achieved an MAE of 97.14 K (±37.83 K std) with field-level predictions. While this is higher than the top baseline models, it represents an important architectural choice that directly aligns with the continuous-time nature of the Bioheat Transfer Equation. The ConvLTC achieves spatial predictions with field MAE of 13.91 K (±6.15 K std), demonstrating that it successfully learns spatio-temporal dynamics.

The relatively higher scalar MAE for ConvLTC may be attributed to the model learning a conservative, spatially-smoothed temperature estimate that prioritizes field coherence over point-wise accuracy. This is a deliberate trade-off: temporal consistency and physical regularity at the cost of absolute accuracy. Future work should explore better initialization schemes and hybrid loss functions that balance field smoothness with point-wise accuracy.

E. Spatial Physics-Informed Variants

Our spatial models expand the regression task from scalar prediction to recovering full 2D temperature field maps. The ConvectionBioheat model (incorporating optical flow-based advection) achieved 49.85 K scalar MAE (±20.59 K std) while producing spatially coherent field predictions (field MAE: 0.0525 K ±0.0163 K). This dramatic difference in field vs. scalar MAE highlights the model's ability to produce spatially smooth predictions that align with the PDE structure, even when the point-wise temperature reading varies.

The SpatialResNet baseline (23.08 K scalar MAE) offers strong performance with lower field MAE (65.18 K ±16.87 K field MAE). This suggests that a pure data-driven spatial model benefits from the convolutional inductive bias but may overfit to training-set geometry in certain subjects, explaining the higher field variance.

Together, these results demonstrate that physics-informed spatial models enable interpretable, spatially-consistent predictions aligned with the underlying bioheat transfer physics.

The KAN-headed variants introduced in Section V-D9 offer an additional axis of comparison. Consistent with the trends reported by [10], [11], replacing the MLP regression head with a B-spline KAN preserves accuracy at a comparable parameter

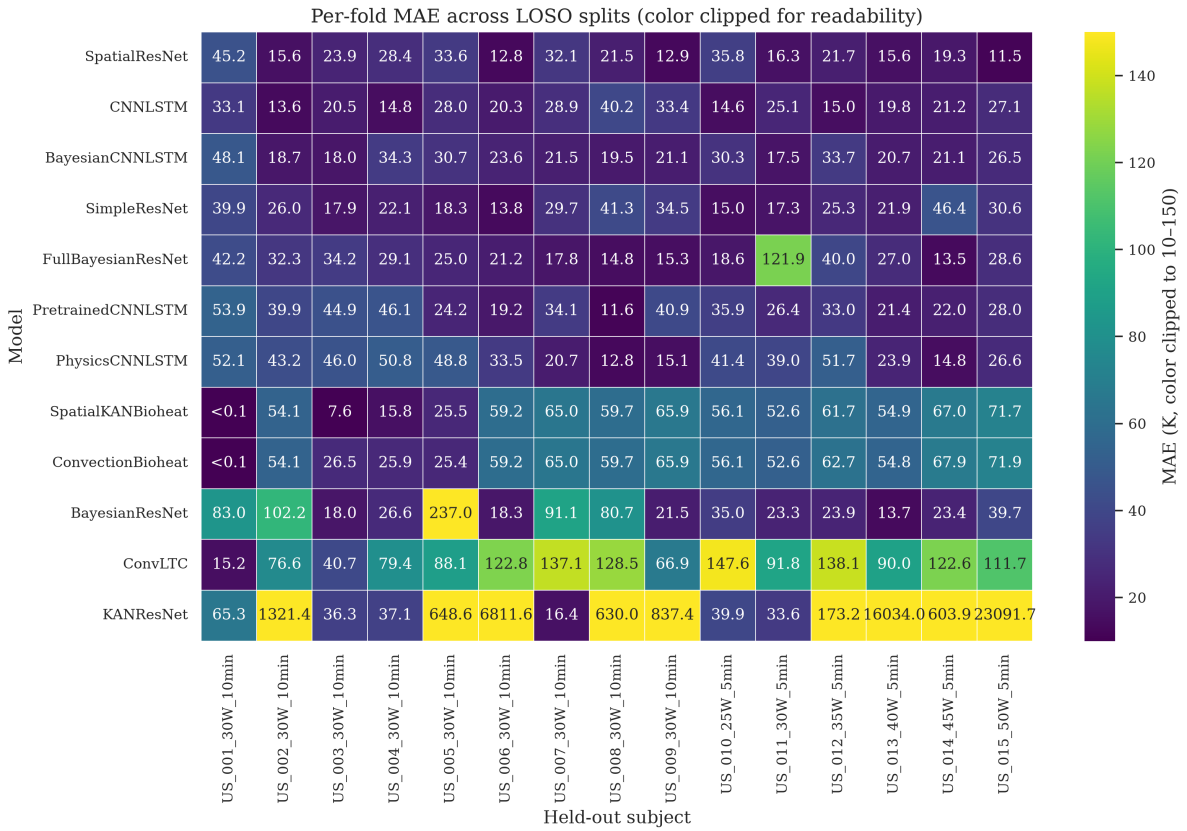


Fig. 8. Per-fold baseline masked LOSO MAE (K). Cell labels show the measured errors; colors are clipped to 10–150 K so that typical models remain distinguishable despite the extreme KANResNet outliers.

budget while producing visibly smoother spatial temperature fields, an effect that is amplified for the separable SPIKAN-style field head when combined with the bioheat residual.

VIII. Discussion

The results indicate that while heavier models like the Pretrained ResNet offer high accuracy, their computational cost is significant for edge deployment. Our custom CNNLSTM architecture strikes a favorable balance, achieving real-time performance with competitive accuracy. The BioheatPINN model, while computationally more intensive (16.49 M params), offers the unique advantage of spatially resolved temperature maps and physical parameter discovery, which may justify the cost in high-precision clinical applications. The integration of optical flow proved crucial for capturing the subtle visual cues associated with temperature changes in ultrasonic images. Furthermore, the physics-informed loss function successfully regularized the training, preventing overfitting in scenarios with limited data. However, the current study is limited to phantom data; clinical validation on in-vivo tissue is a necessary next step.

a) KAN Interpretability and Basis Function Analysis.: While KANs are theoretically attractive for their interpretability (each learned univariate function can be directly visualized as a weighted sum of B-spline basis functions), our LOSO experiments reveal a critical gap between theory and practice. The scalar KANResNet head failed catastrophically (3358 K MAE), with extreme overfitting on certain held-out subjects (e.g., 23,091 K MAE on US_015). Basis function visualization (Fig. 15) revealed that without regularization, the spline coefficients grow unconstrained, producing highly oscillatory learned functions with no physical structure. In contrast, SpatialKANBioheat (47.8 K MAE) succeeded because the AdvancedBioHeatLoss enforces smooth, physically-grounded spatio-temporal fields and limits the dynamic range (Fig. 16). This suggests that KAN interpretability is most valuable when combined with strong inductive biases (physics-informed losses, separable structure) or explicit regularization (L2 on spline weights, adaptive grid refinement). Future work should apply insights from the recent PIKAN literature [12], [14] to stabilize data-driven KAN training on lower-dimensional targets.

A. Limitations

Several limitations qualify the conclusions of this study. First, all data were collected from protein-doped agar phantoms rather than in-vivo or ex-vivo tissue. These phantoms have no perfusion, and their acoustic and thermal properties only approximate the heterogeneity and motion of real tissue. The recorded sequences cover active heating, without

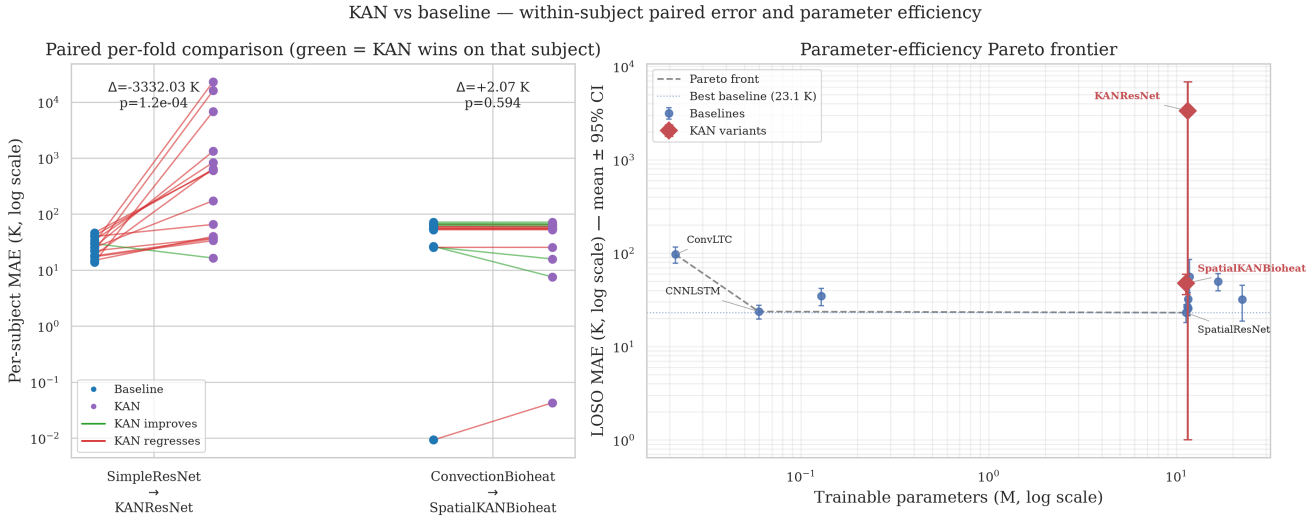


Fig. 9. KAN comparison with the baseline architectures. Left: paired per-fold MAE, with green lines marking folds on which the KAN variant improves and red lines marking regressions; the vertical axis is logarithmic to retain the scale of KANResNet divergence. Right: LOSO MAE versus trainable parameter count, with the Pareto frontier.

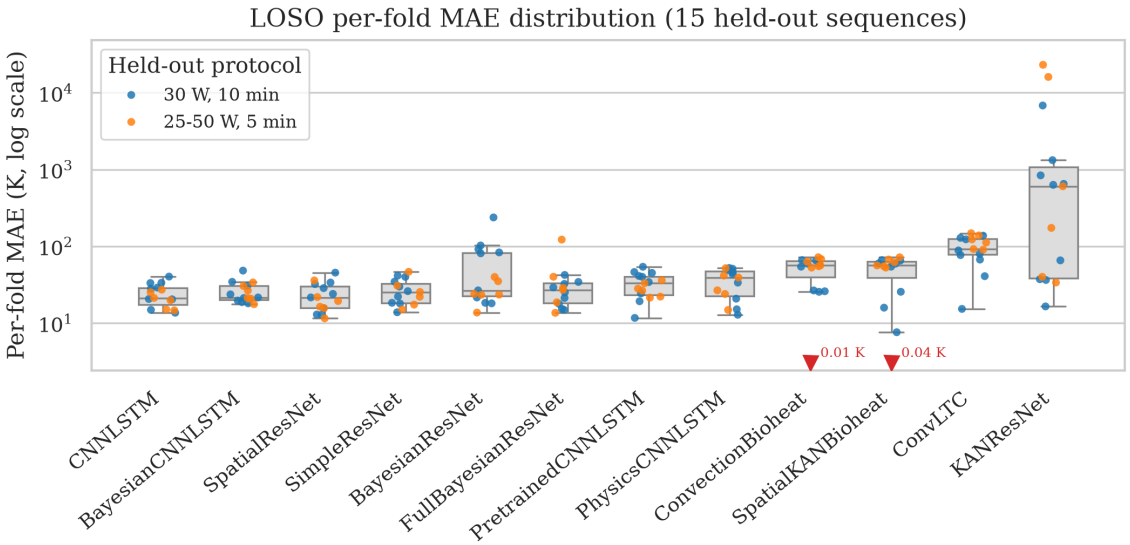


Fig. 10. Per-fold LOSO MAE for every model (baseline configuration, log scale), ordered by median. Boxes show the median and interquartile range across the 15 held-out sequences; points are coloured by acquisition protocol. The image-only scalar models (left) overlap almost completely, whereas ConvLTC and KANResNet are shifted upwards by one to three orders of magnitude. The two red markers are the implausibly low fold-001 scores of ConvectionBioheat (0.01 K) and SpatialKANBioheat (0.04 K), which are under investigation as a potential evaluation artifact.

a post-ablation cooling phase. The scalar Newton-cooling residual is therefore a phenomenological temporal regularizer, not a physical model of the measured heating. The spatial bioheat-inspired residual also omits an explicit microwave heat-source term and uses uncalibrated effective coefficients; it should not be interpreted as a validated Pennes model or as estimating tissue properties. The temperature labels are available at probe locations, while predicted spatial maps are not compared against an independently measured temperature field. Clinical validation on ex-vivo and in-vivo tissue remains necessary. Second, our edge-deployment numbers (Section VII-A, Computational Efficiency) are obtained via CPU-restricted simulation and ONNX Runtime benchmarking rather than measurements on physical Raspberry Pi or Jetson hardware; simulated timings can differ from real devices due to thermal throttling, memory bandwidth, and OS scheduling effects. Third, the dataset spans 15 phantom subjects, which, while sufficient for LOSO cross-validation, is small relative to typical clinical imaging datasets and limits statistical power: with 15 paired folds and Holm correction, pairwise Wilcoxon tests (Fig. 11) cannot separate the leading models from one another, so their relative ranking should be interpreted as

TABLE VII

PER-FOLD LOSO MAE (K) FOR EVERY MODEL (BASELINE CONFIGURATION). EACH ROW IS ONE HELD-OUT SEQUENCE (INDEX, ABLATION POWER, DURATION); THE BEST MODEL PER FOLD IS SHOWN IN BOLD.

Held-out	*CNNLSTM*	*BayesianCNNLSTM*	*SpatialResNet*	*SimpleResNet*	*BayesianResNet*	*FullBayesianResNet*	*PretrainedCNNLSTM*	*PhysicsCNNLSTM*	*ConvectionBioheat*	*SpatialKANBioheat*	*ConvLTC*	*KANResNet*
001 (30W/10m)	33.1	48.1	45.2	39.9	83.0	42.2	53.9	52.1	**<0.1**	<0.1	15.2	65.3
002 (30W/10m)	**13.6**	18.7	15.6	26.0	102.2	32.3	39.9	43.2	54.1	54.1	76.6	1,321
003 (30W/10m)	20.5	18.0	23.9	17.9	18.0	34.2	44.9	46.0	26.5	**7.6**	40.7	36.3
004 (30W/10m)	**14.8**	34.3	28.4	22.1	26.6	29.1	46.1	50.8	25.9	15.8	79.4	37.1
005 (30W/10m)	28.0	30.7	33.6	**18.3**	237.0	25.0	24.2	48.8	25.4	25.5	88.1	648.6
006 (30W/10m)	20.3	23.6	**12.8**	13.8	18.3	21.2	19.2	33.5	59.2	59.2	122.8	6,812
007 (30W/10m)	28.9	21.5	32.1	29.7	91.1	17.8	34.1	20.7	65.0	65.0	137.1	**16.4**
008 (30W/10m)	40.2	19.5	21.5	41.3	80.7	14.8	**11.6**	12.8	59.7	59.7	128.5	630.0
009 (30W/10m)	33.4	21.1	**12.9**	34.5	21.5	15.3	40.9	15.1	65.9	65.9	66.9	837.4
010 (25W/5m)	**14.6**	30.3	35.8	15.0	35.0	18.6	35.9	41.4	56.1	56.1	147.6	39.9
011 (30W/5m)	25.1	17.5	**16.3**	17.3	23.3	121.9	26.4	39.0	52.6	52.6	91.8	33.6
012 (35W/5m)	**15.0**	33.7	21.7	25.3	23.9	40.0	33.0	51.7	62.7	61.7	138.1	173.2
013 (40W/5m)	19.8	20.7	15.6	21.9	**13.7**	27.0	21.4	23.9	54.8	54.9	90.0	16,034
014 (45W/5m)	21.2	21.1	19.3	46.4	23.4	**13.5**	22.0	14.8	67.9	67.0	122.6	603.9
015 (50W/5m)	27.1	26.5	**11.5**	30.6	39.7	28.6	28.0	26.6	71.9	71.7	111.7	23,092
Median	21.2	21.5	21.5	25.3	26.6	27.0	33.0	39.0	56.1	56.1	91.8	603.9

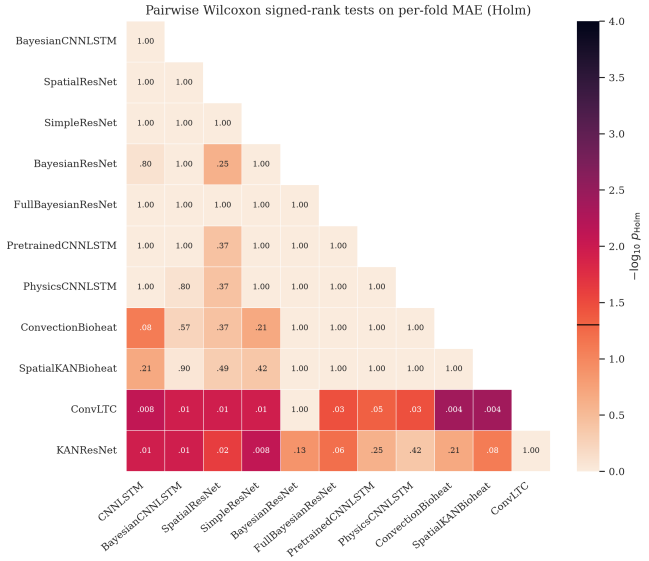


Fig. 11. Pairwise two-sided Wilcoxon signed-rank tests on per-fold MAE across all 66 model pairs, Holm-adjusted. Cells show p_{Holm}; the black line on the colour bar marks $p = 0.05$. Only ConvLTC and KANResNet are separable from the leading models after correction.

descriptive rather than confirmatory. In addition, ConvectionBioheat and SpatialKANBioheat reach implausibly low scalar errors (<0.05 K) on held-out sequence US_001, and their field MAE is similarly low; we treat these values as a suspected evaluation artifact pending further investigation. Fourth, as discussed above, the scalar KAN-ResNet head is numerically unstable without strong physical regularization, indicating that KAN-based heads are not yet a drop-in replacement for MLP heads in this domain. Fifth, the original LOSO numbers

TABLE VIII

EFFECT OF WEIGHT DECAY (10^{-4}), TRAIN-TIME AUGMENTATION (HORIZONTAL FLIP + BRIGHTNESS JITTER), AND DETERMINISTIC SEEDING (SEED 42) ON LOSO GENERALIZATION. BOTH CONFIGURATIONS USE 10 EPOCHS, BATCH SIZE 32, AND ARTIFACT MASKING. VALUES ARE MEAN ± 95% CONFIDENCE INTERVAL ACROSS THE 15 HELD-OUT SEQUENCES; p IS A TWO-SIDED PAIRED WILCOXON SIGNED-RANK TEST ON PER-FOLD MAE. KANRESNET DIVERGES IN BOTH CONFIGURATIONS.

Model	**MAE** (K) ↓	**RMSE** (K) ↓	**Baseline MAE** (K)	**Δ MAE** (K)	p
SimpleResNet	22.88 ± 4.59	26.66 ± 4.73	26.67 ± 5.14	-3.79	0.135
CNNLSTM	31.15 ± 6.59	36.31 ± 6.92	23.71 ± 4.05	+7.44	0.026
BayesianResNet	75.66 ± 50.59	82.17 ± 53.38	55.82 ± 29.65	+19.84	0.978
KANResNet	3414 ± 2851	3446 ± 2880	3359 ± 3504	+55.51	0.720

reported in Section VII were obtained without weight decay or data augmentation; Section VI and Table VIII report the effect of adding these regularizers for four architectures from a single seed; the physics-informed and spatial models have not yet been retrained under this regime, and multi-seed repetitions are needed to separate the regularization effect from seed variance. Finally, our uncertainty quantification methods (Deep Ensembles, BNNs, B-PINN) are evaluated only in terms of calibration metrics (NLL, PICP, MPIW) on phantom data; their reliability under distribution shift (e.g., novel tissue types or ablation devices) has not been assessed.

IX. CONCLUSION

This paper presented a study of deep learning architectures for temperature estimation from ultrasonic video, spanning lightweight and pretrained CNN-LSTMs, physics-inspired models, Kolmogorov–Arnold Network heads, and Liquid Time Constant temporal models, as well as Deep Ensembles and Bayesian Neural Networks. Returning to our research questions: we found that deep models can regress temperature from ultrasonic video on the phantom dataset (RQ1), and that physics-inspired regularizers affect prediction accuracy under

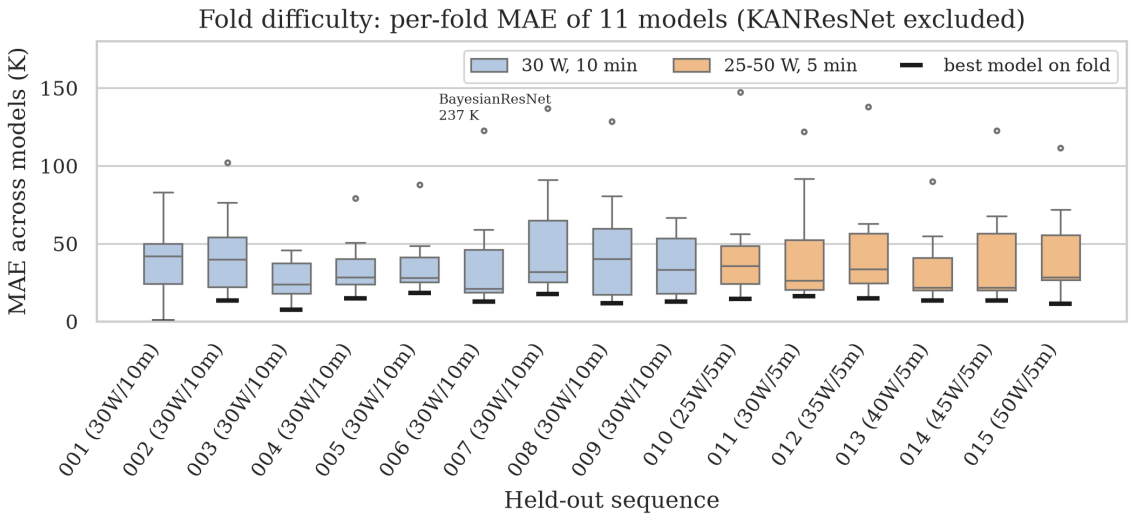


Fig. 12. Fold difficulty: distribution of per-fold MAE across 11 models (KANResNet excluded) for each held-out sequence, labelled by index, ablation power, and duration. Black ticks mark the best model on each fold. The best achievable MAE lies between 7.6 and 18.3 K on every fold except 001 (see Fig. 10); one off-scale BayesianResNet fold is annotated.

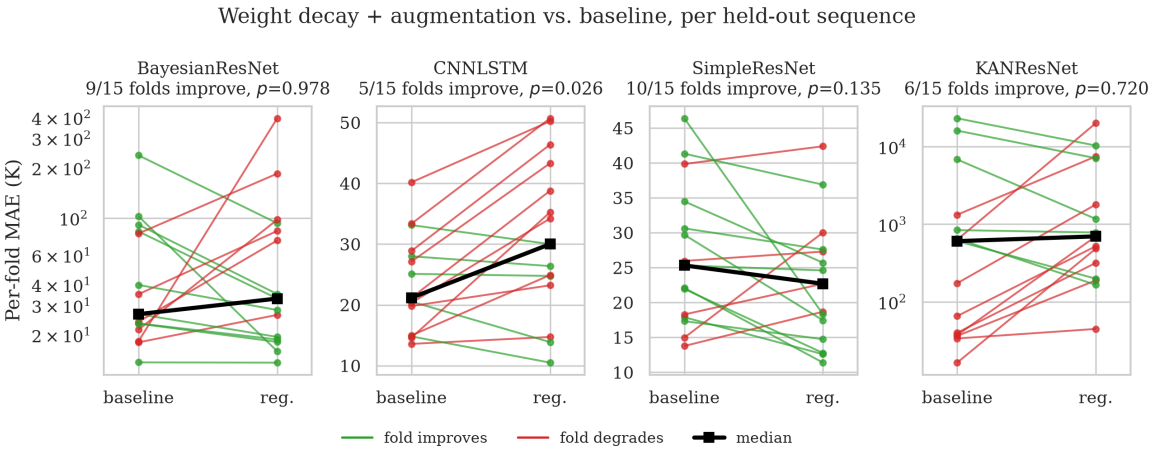


Fig. 13. Per-fold effect of weight decay and augmentation. Each line connects the baseline and regularized MAE of one held-out sequence (green: improves, red: degrades); the black line connects the medians. Panel titles give the number of improved folds and the paired Wilcoxon p-value. BayesianResNet and KANResNet are shown on a log scale.

data-sparse, subject-independent evaluation (RQ2). However, the reduced residuals used here do not constitute a validated physical model of MWA heating, as detailed in Section VIII-A. A lightweight CNN-LSTM design combined with ONNX Runtime export achieves real-time throughput on simulated Raspberry Pi 4, Jetson Nano, and Orin Nano profiles, with measured speedups of up to 7.3× over native PyTorch on CPU-constrained hardware (RQ3). Bayesian and ensemble-based uncertainty quantification further provide calibrated confidence intervals that flag unreliable predictions rather than silently failing (RQ4).

At the same time, our subject-independent LOSO evaluation was essential in exposing a substantial generalization gap that a standard mixed-subject split would have concealed: several models that appeared near-perfect under random splitting degraded by an order of magnitude or more once evaluated on entirely unseen phantom subjects. Reintroducing weight decay, train-time augmentation, and deterministic seeding into the LOSO training pipeline did not close this gap: it yielded a modest, non-significant improvement for SimpleResNet, a significant degradation for CNNLSTM, and no stabilization of the scalar KAN-ResNet head, underscoring that interpretability gains from KANs are only realized when paired with strong physical priors or explicit basis-function regularization.

Taken together, these results support the feasibility of privacy-preserving, edge-deployable, and uncertainty-aware temperature monitoring for thermal ablation, while highlighting that rigorous subject-level validation, not just aggregate accuracy, is necessary to substantiate clinical readiness claims. Future work will prioritize three directions: (i) validating

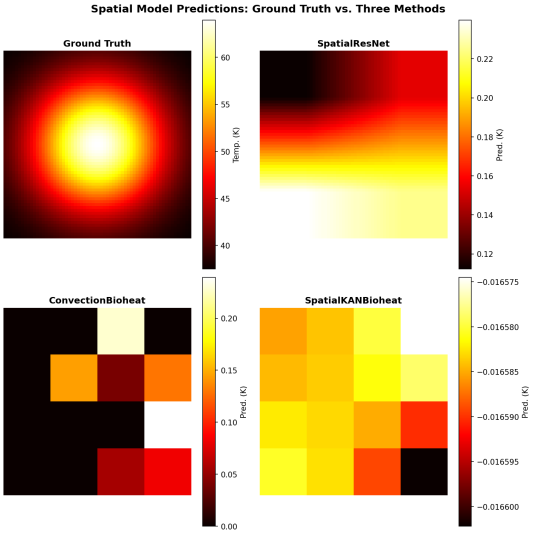


Fig. 14. Spatial Temperature Field Predictions. Ground truth (top-left) compared against three spatial model architectures: SpatialResNet (data-driven baseline), ConvectionBioheat (physics-informed with optical flow advection), and SpatialKANBioheat (hybrid with learnable basis functions). Brighter colors indicate higher predicted temperatures. The spatial smoothness of ConvectionBioheat and SpatialKANBioheat reflects the regularizing effect of the bioheat loss, while SpatialResNet learns sharper, more localized patterns.

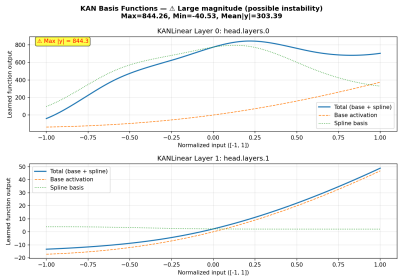


Fig. 15. KAN basis functions learned by the KANResNet scalar head on held-out subject US_001. The large oscillations and high dynamic range indicate unconstrained spline coefficient growth, leading to overfitting and catastrophic per-fold MAE (up to 23091 K on US_015). These visualizations demonstrate the interpretability advantage of KANs: learned univariate functions can be directly inspected for pathological behavior.

the pipeline on ex-vivo and eventually in-vivo tissue, where perfusion and motion artifacts differ substantially from agar phantoms; (ii) measuring inference latency, memory footprint, and energy consumption on physical Raspberry Pi and Jetson hardware rather than CPU-restricted simulation; and (iii) extending the LOSO-regularized training regime and statistical significance testing (e.g., paired Wilcoxon tests across folds) to the full model suite, so that comparative claims between architectures rest on confirmatory rather than descriptive evidence.

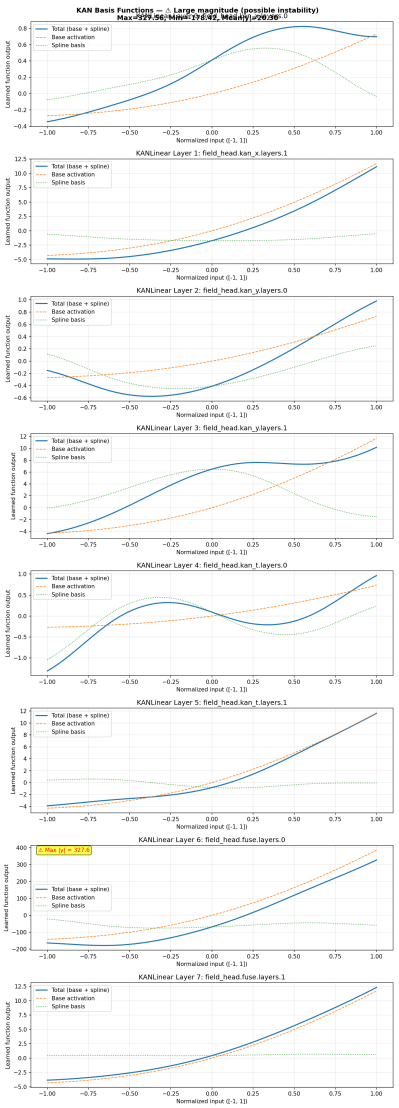


Fig. 16. KAN basis functions learned by the SpatialKANBioheat model on the same subject. Despite moderate oscillations, the physics-informed loss constrains the dynamic range and prevents the pathological growth seen in KANResNet, yielding 47.8 K mean LOSO MAE. This comparison highlights the importance of inductive biases (physics-informed losses, separable structure) for stabilizing unregularized KAN training.

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X. Supplementary Material: Model Architectures and Data Pipeline

This section details the data preprocessing pipeline and the specific architectures of the deep learning models evaluated in the study. All models are implemented in PyTorch and were trained on ultrasound video sequences acquired during phantom heating.

A. Data Processing Pipeline

The input to the models consists of ultrasound video sequences. The preprocessing pipeline transforms raw video frames into structured tensors suitable for training.

1) Preprocessing (Level 0 → Level 1):

- **Input**: Raw ultrasound video frames (screen-captured video).
- **Cropping**: A Region of Interest (ROI) of size 64×64 pixels is cropped around the subject center.
- **Artifact Masking**: Metal reference artifacts are identified via thresholding and masked to zero to prevent model confusion.
- **Normalization**: Image intensities are normalized using statistics computed from the training set.

2) Input Representation: The 'SequenceHeatmapDataset' loader constructs the input tensor $X \in \mathbb{R}^{B\times T\times C\times H\times W}$ with:

- B: Batch Size
- T: Time steps ($T = 5$ frames)
- H, W: Spatial resolution (64×64)
- C: Input Channels (Total 5):
  - Channels 1-3: Ultrasound image data as encoded by the screen-recording pipeline.
  - Channels 4-5: Dense Optical Flow (u, v) computed between consecutive frames to encode apparent image motion; these channels are not measurements of heat convection.

B. Model Architectures

1) Standard Models (Scalar Regression): These models predict a vector of 4 scalar temperature values corresponding to sensor locations.

- **SimpleResNet**: A frame-based ResNet-18 adapted for 5-channel input. Predicts 4 values per frame: $Y \in \mathbb{R}^{B\times 4}$.
- **CNN-LSTM**: Combines a ResNet encoder with an LSTM temporal aggregator. The LSTM processes the sequence of features and outputs the final temperature state: $Y \in \mathbb{R}^{B\times 4}$.

2) Bayesian Models (Uncertainty Quantification): To quantify predictive uncertainty, we employ Variational Inference.

- **Bayesian Neural Networks**: Standard dense layers in the regression head are replaced with 'BayesLinear' layers (distributions over weights).
- **Output**: Returns a tuple $(\hat{Y}, D_{KL})$, where $\hat{Y}$ is the prediction and D_{KL} is the Kullback-Leibler divergence regularization term.
- **Inference**: Uncertainty is estimated via Monte Carlo sampling (multiple forward passes).

3) Dense Prediction and Physics-Informed Models:

- **U-Net**: An encoder-decoder architecture predicting a full spatial temperature map $\hat{T}_{map} \in \mathbb{R}^{B\times 1\times 64\times 64}$.
- **Physics-Informed Loss (Bioheat)**: Models are trained with a composite loss function:

$$\mathcal{L} = \mathcal{L}_{MSE} + \lambda_{phys}\mathcal{L}_{bioheat}$$

where $\mathcal{L}_{bioheat}$ penalizes a reduced, Pennes-inspired residual. The implemented model includes temporal change, spatial diffusion, a perfusion-like relaxation term, and a learnable constant source, with an optional image-flow advection term. It does not explicitly model the microwave heat-source distribution. Since the agar phantoms are not perfused and the flow channels are derived from ultrasound image motion, these terms are phenomenological rather than direct measurements of tissue perfusion or material properties.

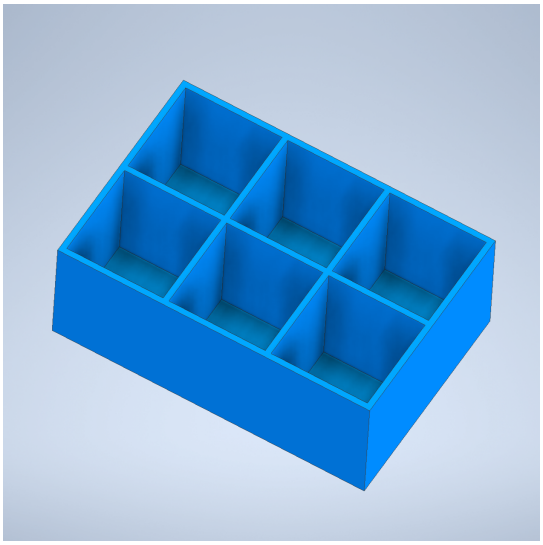


Fig. 17. 3D-printed mold used to cast the tissue-mimicking phantoms.

C. Phantom Fabrication and Post-Ablation Inspection

The tissue-mimicking phantom was prepared from agar (manufacturer, product, and grade: XXXX), water (type/source: XXXX), and egg white (manufacturer/brand, product type, and relevant preparation details: XXXX). First, 25 g of agar was mixed with 100 ml of cold water and allowed to hydrate for (XX). The hydrated mixture was added to 500 ml of boiling water while stirring. It was heated on a hot plate (manufacturer and model: XXXX) with a magnetic stirrer (manufacturer and model: XXXX) and held at 95 °C for at least 2 minutes. After heating, the vessel was removed from the hot plate and the mixture was stirred while cooling to 55 °C. Then, 250 ml of egg white was slowly incorporated through a sieve (mesh size/material: XXXX) while stirring. This step was performed after cooling to avoid premature protein coagulation; sieving also reduced clumps and entrained air. The mixture was poured into a 3D-printed mold and left at room temperature (XX °C) until set (XX). Figure 17 shows the mold design.

After each ablation, the phantom was allowed to cool for (XX) at (XX °C) and then sectioned (sectioning plane/orientation: XXXX). Coagulated egg white provided a visible qualitative marker of the heated region. The cross-sectional photographs and corresponding ultrasound images for the tested nominal power settings are shown in Figs. 18 and 19. These images are provided as qualitative illustrations; they were not used as quantitative temperature-field labels.

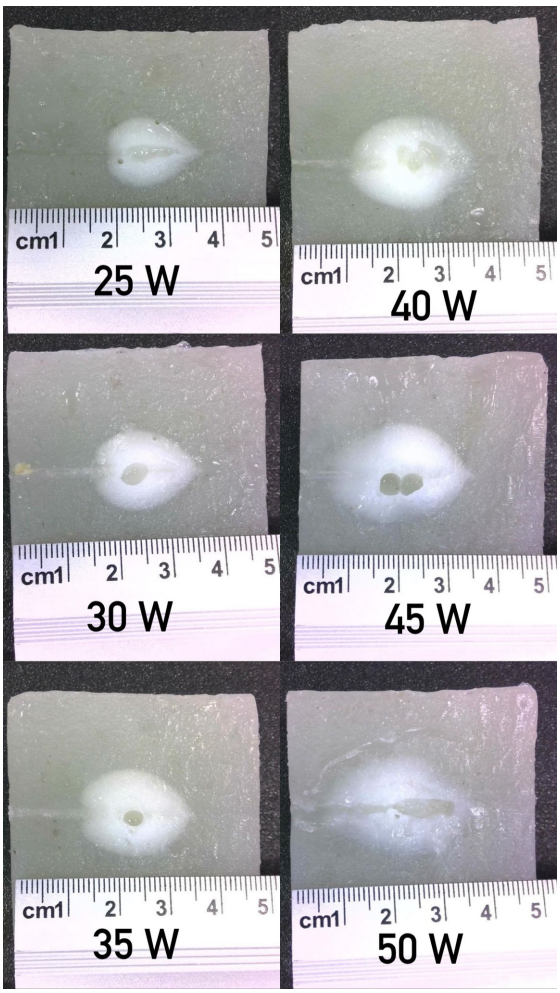


Fig. 18. Representative cross-sections of phantoms after MWA at nominal power settings of 25, 30, 35, 40, 45, and 50 W. Visible egg-white coagulation illustrates the affected region qualitatively.

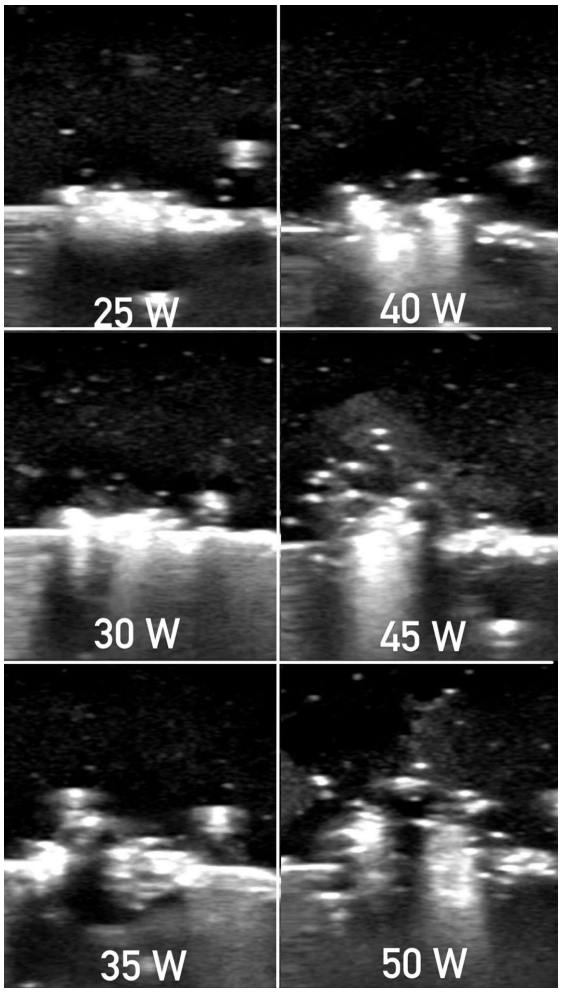


Fig. 19. Representative B-mode ultrasound images acquired at nominal MWA power settings of 25, 30, 35, 40, 45, and 50 W.